

CHAPTER ONE

Is This a Phase?

AN EXCERPT FROM

God Makes No Mistakes

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T L D R

Every parent of a transgender child asks this question. So did I, for longer than I should have. This chapter will not tell you that your doubt is wrong. It will give you something more useful: a framework for observing your child honestly over time, rather than reacting to a conclusion you have already reached. The false positive problem is real. So is the cost of treating a persistent identity as a passing trend. This chapter shows you how to tell the difference, names the fear most parents will never say out loud, and asks you to look clearly at the four possible futures in front of you, because only one of them ends well for everyone.

I was absolutely certain it was a phase.

Not defensively certain. Not the kind of certainty that covers for doubt underneath. I mean the relaxed, unexamined certainty of a man who has already seen this movie as the protagonist or as a witness. Kids go through things. I went through things. I had a ponytail down to my shoulders at fifteen, shredded jeans like Axl Rose from Guns N' Roses, a communist phase at university that lasted nearly two years before the intellectual exercise, the evidence and the deep understanding that my economics degree demanded finally dismantled it. My father watched all of it from a respectful distance. He never said you're being ridiculous, this is just a phase. He said: observe it. You're intelligent enough to think for yourself.

I thought I was doing the same thing for Valentin. Giving him space. Being modern about it. Not panicking.

What I was actually doing was deciding in advance that this would somehow pass and watching through that lens to find evidence that confirmed what I already "knew": that this will pass.

That is not observation. That is the illusion of observation.

The first conversation happened over the phone. Valentin told me he felt non-binary. He was around twelve at the time. I had never encountered the word used that way. I asked him, genuinely, what it meant, not to challenge him, but because I truly had no idea. I had no framework, no definition to receive it. He explained clearly and patiently. I listened. I said I needed to think about it and do some research.

I researched for weeks. Read everything I could find. Still couldn't make it land in any architecture I possessed. I understood what the words were saying, but it didn't register in my mind. My categories were binary: male, female. Two chromosomes, two outcomes, two life paths. What Valentin was describing didn't fit anywhere in that structure. Furthermore, I could not imagine the feeling, the thought of being non-binary.

So I did what people do when new information won't fit the existing framework. I kept the framework and reinterpreted the information.

This is a phase, I told myself. Adolescence is brutal. The internet is full of things that make kids who are absorbing an immense amount of information confused. He's in a new country, no real friends, a school that has been making his life miserable. Of course he is reaching for an identity. Of course he is experimenting. That is what twelve-year-olds do. I did it. Every person I know did it. The ponytail grew out. The jeans got replaced. This would pass too.

Eight months later, Valentin told me he was a man. He had, he said, felt this way since he was four or five years old.

I want to stop here and be honest with you, because you are reading this book precisely because perhaps you are standing where I was standing, and what I did next cost us both something we never fully get back.

I did not adapt my response when he told me he was a man. I did not see the progression from non-binary to a clear male identity as evidence that this was deepening rather than fading. I took both data points and reassembled them into the

same conclusion: still a phase, now a more serious phase, will still pass.

Meanwhile, Valentin was being bullied at school. He had no friends. He had lost his hometown city, his language, his grandmother and aunt, everything familiar, all at once. He was in therapy, which helped the way a bandage helps internal bleeding. He had started painting in red and black, figures in distress, dark and intense, and the school had interpreted a suffering child's art as a threat to other students. The bullies were, in the eyes of the administration, defending themselves from my son.

I was in Barcelona at the time. He was in Santo Domingo de la Calzada with his mother. Santo Domingo de la Calzada is a small rural town in Spain with a population of 6,000 whose only merit is being part of el camino de Santiago and has a beautiful cathedral. I wasn't there to truly understand the situation as I didn't live there. I could not see any of this with my own eyes. I was receiving reports, filtering them through the phase theory, concluding each time that things were difficult but manageable.

They were not manageable. They were catastrophic. I just couldn't see it from where I was standing.

The question your skeptical friend is actually asking

Before I give you the framework I eventually built, I want to address the version of this question that comes from the most serious critics of the trans movement. Because they are not all wrong, and dismissing them does not make the concerns disappear.

A good friend of mine for over 35 years, a highly intelligent, data-driven engineer and statistician consultant, the kind of man who reads philosophy and builds organisational models and doesn't make claims without being able to source them, put it to me directly. He has a young daughter. He is watching this conversation unfold in the culture and he is worried. He believes social contagion is real. He believes the incidence of false positives is significant and systematically underreported. He has read many research papers including the long-term transition regret studies out of Sweden and the UK. He has seen the Cass Review. He believes there is an unofficial clinical and social script that moves children toward affirmation faster than the evidence warrants.

He is not a bigot. On the contrary, he's one of the most intellectually open-minded people I've come to know. He is not cruel nor superficial. He is a father looking at a landscape he finds genuinely alarming.

He deserves a real answer, not a dismissal.

So here is my honest assessment as a father who has researched this deeply:

On social contagion: the evidence that peer influence creates gender dysphoria in children who were otherwise developing as cisgender is weak. What the evidence does support is that increased visibility and social acceptance has reduced the age at which children feel safe enough to express and name what they were already experiencing. Those are different phenomena. But the rapid increase in adolescent identification, particularly among teenage girls, is not something serious researchers wave away, and pretending the pattern doesn't exist doesn't serve anyone. [53]

On false positives: they do exist and are real. Some people transition and later change course. The research on detransition is underdeveloped and politically contested, which means the honest answer is that we don't have clean numbers. What we do have is a consistent signal that regret is more common among people who transitioned rapidly, without parental and psychological support, without the long observation period that careful parenting and clinical practice requires. That is an argument for a careful process, not a blanket refusal. [54]

On the long-term outcome data: studies showing significant wellbeing improvements after transition consistently outnumber those showing the opposite. The Swedish and UK data cited in detransition arguments are real but frequently stripped of context. The Swedish cohort, for example, had transitioned with far less social support than current patients receive. Methodology matters enormously here, and anyone telling you the science is settled in either direction is selling you something. [55]

This said, my friend's concerns do not add up to refusing to accept your child's identity. They add up to something worth taking seriously. The pace and certainty of your response matters. Observation over time matters. The difference between a supportive parent and a compliant one is the difference between watching carefully and abdicating judgment entirely.

What persistent actually looks like

Clinical practice in paediatric gender medicine uses a framework simpler than most people expect.

A child's gender identity is more likely to be persistent, rather than exploratory, when several things are true across time. The identity is consistent: the child describes it the same way in different contexts, with different people, across months and years. It is insistent: the feeling is stated with certainty rather than offered as a question. It is present from early childhood rather than appearing suddenly after social exposure. It resists gentle redirection, not because the child is stubborn, but because there is nothing to redirect. The distress comes from the mismatch between the child's identity and how the world sees them, not from the identity itself. [56]

None of this constitutes a guarantee. It is a framework for observation, not a diagnostic test.

Valentin's pattern, looked at honestly, hit every marker. He had felt this way since age four or five. He had performed the role assigned to him for years because he did not know what else to do. He named it the moment he found language and the courage to express it. The consistency across time, from non-binary at twelve to a clear male identity eight months later to absolute certainty twelve years after that, is not the pattern of a phase. Phases fade away. This deepened.

What I missed was that the progression from non-binary to male was not confusion becoming more confusion. It was a child finding more precise language for something that had always been true. That distinction took me too long to understand.

I will also tell you this: for the first six months to a year after he told me he was a man, part of my mind was still constructing an external explanation. Influencers. YouTubers. The trans community was a kind of ideological safe house that had given Valentin a framework for interpreting something that wasn't really there. That interpretation was comfortable because it made the problem external. Fixable. Something outside the relationship rather than inside it.

It was also wrong. The part of me trained since university to follow evidence rather than noise eventually forced me to admit that.

The fear I didn't say out loud

Let me tell you something I have not said out loud before, because I think most parents in this situation have had the same thought and none of them want to admit it.

When Valentin started self-harming and began expressing suicidal thoughts, even then it crossed my mind that this might be a manipulation tactic. A way to force my compliance with something I had categorised in my own head as a delusion.

As parents we've seen this happen time and again for years, tantrums and continuous boundary testing that required our steady hand and firm resolution to prevent our kids from being spoiled rather than growing resilient.

I am saying this plainly. It crossed my mind.

I had always believed, without ever examining the belief, that suicidal thoughts were a feature of weak and faithless people. They lacked the willpower to navigate what life put in front of them and did not trust God's benevolent plan. I had my own difficult moments: I've been bankrupt, heartbroken and betrayed, almost killed twice by the dictatorship in Venezuela, forced into exile and starting from zero, depressed, lived through the passing of my beloved father and so much more that I could write a short book about it. You could too, I'm sure. But I had always pushed through, because that is what I believed strong people did, and yes, I'm Gen X. So when Valentin expressed that life was unbearable, that he couldn't find joy, that he didn't want to live, part of my response was a judgment. I never named it directly but it was present in our conversations indirectly. I thought: you are reaching for a weapon because you know it is the only thing that will make me move into compliance.

I did not say it out loud. But it was there in how I spoke to him, in the firmness I maintained, in the way I redirected every crisis conversation toward hope, reasons and faith to live rather than toward the thing he was actually trying to tell me that was causing the situation.

This was partly why I pushed so hard for the psychiatric route. My logic, unexamined: I cannot engage with this directly because I am too close to the issue. I will remove myself from the dynamic and let professionals correct the course. I cannot comply

with a delusion on the basis of a child's distress, however real it looks. I had read the detransition research. I knew regret was real. I was going to hold the line. Be a responsible adult. Prevent the future catastrophe.

The medication complied. Valentin's brain slowed down. Self-harming decreased. He was a zombie who could no longer paint, or feel joy, but he was not self-harming. I called that progress.

At the time I was living through an important marital crisis. My wife and I agreed to take a break. I moved in with my brother and his family in Barcelona. So I decided to take a road trip with Valentin along the north coast of Spain, just the two of us: the Cantabric landscape and towns, the Atlantic in November, good food and no agenda.

On this trip he told me that he had kept taking the medication for months after he wanted to stop. He did it because he didn't want to fail me and he trusted me deeply as his guide on earth to know what to do.

He was suffering in silence inside a treatment I had insisted on. He continued because he didn't want to lose my approval, and because he trusted that I loved him infinitely and would always do what was in his best interest. I told him many times throughout his life that as his father that was my commitment, my duty and my joy from the day he was born.

After understanding his reality, how he was feeling disconnected and unbodied, physically alive but spiritually dead, and that he had kept taking the antidepressants, living like a zombie, because he knew I loved him and trusted me to guide him forward, I felt devastated. I realised the extent to which my protective love was damaging my son. Can you imagine a twelve-year-old on antidepressants? I did that. I was the enabler, the "responsible" father. This was a deep regret I carried for a long time, a horrifying shadow that took me years to accept and integrate as part of my personal growth.

Perhaps it was because I was going through my own life crisis, triggered by the separation from my life partner of over fifteen years, that I was able to allow myself to be vulnerable enough to be empathetic and to view the relationship with my son from a different perspective.

I had reflected deeply on the root cause of my separation. I was going to therapy, reading philosophy, listening to podcasts, following spiritual and relationship leaders, taking personal growth courses and working on the priorities in my life. This created a

mental and spiritual focus that demanded extreme ownership of my own feelings, fears and actions. I reframed my life's vision to build a fulfilled and happy life, and that also required reframing my priorities and repairing the damage I had caused to my family and myself.

I was determined to put my family's and my own wellbeing above anything else. Society, church, extended family, friends would have to catch up or simply fade away. As the stoic principle reminds us: *memento mori*. We're all going to die, we don't know when, so I would create and live a joyful life for me and the ones I love, today and every day.

So how did the manipulation theory eventually collapse?

Not because the self-harm stopped. Not because the suicidal thoughts went away. Not because Valentin produced new evidence that convinced me. It collapsed when I stopped asking questions about him and started asking questions about myself.

What kind of environment had I built? What were the actual effects of my decisions, my firmness, my correction, my love expressed through control, on the person I was supposed to be protecting? What did my behaviour look like from his side of the relationship?

When I started doing the internal work, exercising extreme ownership, examining my own shadows, the homophobia and transphobia I had never acknowledged in myself, the inherited mental models I had never questioned, I was no longer able to maintain the interpretation that Valentin was manipulating me. Because what I found when I looked honestly was this: I had built a cage. A loving cage, well-intentioned, built from genuine love. But a cage nevertheless, and my son was dying inside it.

I can't emphasize this enough: my love was real. But the control I was exercising through "love" was suffocating the beloved child it was meant to protect.

The lever, if you insist on that word, is not manipulation. It is the lever of a drowning person grabbing at anything within reach. The question is not whether the suffering is manufactured. The question is what the suffering is a response to. A child who is not seen, not believed, not accepted by the person whose unconditional love they need most will suffer. Not as a strategy. As a consequence.

The catastrophic future I was trying to prevent

This is a crucial aspect I need to name, because it was driving my behaviour the entire time without me being fully conscious of it.

I had a vision, an expectation of what Valentin's life should look like and the part I had to play in order for that future to happen: a successful career, a loving and fulfilled relationship, his ability to be independent and support himself, a critical mind that would allow him to navigate life on his own terms in the ever-changing environment of our world. We parents have a vision for our kids whether consciously or not, and fear, whether we know it or not, drives our decisions.

In my view at the time, accepting the transition would make achieving that vision almost impossible.

How could he live a fulfilled life? Who would love him as a transgender man? How can a "freak," a product of a pro-communist media brainwash movement, the so-called gender ideology, fully develop in this dog-eat-dog world? He would be making his life much more difficult. It is difficult enough without having to add an additional layer of dead weight to his future. He would be marginalized and stigmatized like the trans people I visited at the "zoo" in my teenage years, described in the previous chapter.

Not to mention that if this is a phase, and I was certain it was a phase, then he would regret the decision and I would have been the compliant father who allowed it. Those were no more than fears, and I would not let that happen. Not on my watch.

I was living in fear and anxiety from the uncertainty of a future completely outside my script, my upbringing and my understanding of the world, anticipating a catastrophic outcome that had not arrived and probably never would.

Every decision I made was in service of preventing that outcome. The firmness. The antidepressants. The refusal to fully accept what he was telling me. All of it was downstream of a predicted future I was treating as near-certain.

Why? Because I knew better. I had wised up. I was not naive, and I could spot a threat when I saw it. I had lived long enough to understand what was really going on.

During my research I came across an enlightening reality: modern psychology, neuroscience and philosophical schools confirm that our mind has a vast memory and a spectacular imagination. This is the survival mechanism that allowed us to predict and plan for changes in our environment and thrive as a species for thousands of years. But this marvellous human feature is also the source of suffering in two directions: backward, by reliving past events as if they are present, also known as PTSD; forward, by pre-living futures that haven't arrived and inhabiting them as if they already have, also known as anxiety and stress. The mind does not distinguish between past and future. It only exists in the present moment. When you relive a trauma, you suffer it today. If you project a threat into the future and inhabit it fully, your mind suffers that threat today, as though it has already occurred.

Research shows that more than 85% of the things we anticipate with anxiety and stress, the futures we pre-live, the catastrophes we suffer through in advance, never actually materialise. [57]

Think about that as a parent. What are the futures you are suffering right now that will never arrive? What harm are you inflicting today, on yourself, on your child, on the relationship between you, in order to prevent an outcome that has more than an 85% chance of never happening? What is the cost of that prevention, in the present, to the real child sitting in front of you?

I am not saying this to make the decision feel weightless. I am saying it to name the mechanism. A loving parent can cause serious harm while completely convinced they are protecting their child from harm. I know this because I was that parent, and I nearly lost my son not to the catastrophe that was in my mind but to the preventive actions driven from my fear. This distinction matters: the danger I was fighting was imaginary. The damage I was doing was real.

The staged process: observation and accompaniment, the answer to regretted transitions

One thing that should give any careful parent genuine relief is that the medical pathway for gender transition is not a single irreversible decision made in the first six months.

The staged process is not a rushed decision tree. It is a joint journey, led by the child's signals and guided by parents and clinicians, that gives everyone involved the time and information needed to make mature, informed choices.

It opens the door for your child to understand their own reality at their own pace, so that what they ultimately choose comes from clarity rather than desperation.

Not every transgender person transitions in the same way or to the same degree. The process has distinct stages, each with different implications and a different level of reversibility.

This is the clinical framework and what each stage actually means:

Social transition: name, pronouns, clothing, how the child presents in the world. It's fully reversible at any point. Some children try it and step back. That is what reversibility is for.

Puberty blockers: when relevant to the child's timeline, pause puberty rather than redirect it. The effects are largely reversible if the medication is stopped. This is not a minor intervention and requires proper clinical support, but it is not surgery.

Hormone therapy: produces some permanent effects and some reversible ones. It is a gradual process, started at low doses, not undertaken casually and overseen in competent clinical settings.

Surgery: top surgery, bottom surgery, facial procedures. These are permanent. In responsible practice it is not performed on minors and is not the first step in anyone's process. It comes after years of observation and identity confirmation. Many young adults, like Valentin, choose not to pursue surgical intervention at all, or opt for limited procedures like top surgery only.

The regret data that frightens people most comes overwhelmingly from surgical interventions and from processes that lacked adequate time, psychological and parental support, and careful observation over time. The staged approach exists specifically to prevent that outcome. If your child is at the beginning of this journey, the irreversible decisions are many years away. You have time to watch. You have time to ask questions. You have time to be present and discuss rather than be decisive.

What you do not have time for is the alternative: treating a persistent identity as a phase long enough that the cost and psychological burden of your inattention or

hyper-control becomes the child's to carry. I paid that price. You don't have to. That is why you are here reading.

The four futures

I want to lay something out as plainly as I know how to say it, because I think it is the clearest way to show what is actually at stake in the paths in front of you.

There are two variables: what you do, either you treat it as a passing phase or you take it seriously, and what is true about your child, either they are transgender or not. Combined, they produce four possible futures.

Future one: you take it seriously, and it was a phase.

Ten or fifteen years from now you will laugh about it together. You will say: do you remember when you were going through all of that? Your child will say: yes, and thank you. Thank you for loving me through it, for giving me the space to figure myself out, for not making me feel like I had to fight you to be seen. The relationship is intact and it deepens. The trust is intact and growing. You lose nothing except a certainty you didn't need.

Future two: you take it seriously, and it is real.

The same conversation happens, but it carries a different weight. Your child says: thank you for seeing me. Thank you for being beside me through something that was terrifying. Thank you for creating a safe space to experience something that we both had no blueprint, no map for. Thank you for loving me unconditionally and for choosing me over your comfort. The relationship is built on trust, support, admiration and love that will last both your lifetimes.

Future three: you treat it as a phase, and it was a phase.

Perhaps you laugh about it eventually. But your child carries something from that period: the memory of having to fight you to be heard, of having their reality dismissed by the one person who should have believed them first. The relationship survives, but with a scar it didn't need.

Future four: you treat it as a phase, and it is real and permanent.

Two roads. In the better one: a child who spends years in therapy trying to understand why the people who were supposed to love them most could not do it unconditionally, who chose their comfort over their sacred, unspoken promise to be a guide and loving support no matter what. A relationship fractured, perhaps permanently, rebuilt slowly if at all through adult forgiveness and years of therapy.

On the other road: your child becomes part of the statistic. Approximately fifty percent of transgender adolescents consider or attempt suicide, and many of them succeed. [58] I will show you the data in the next chapter. I will also show you the counter-data: research confirming that parental acceptance and affirmation is the single most powerful factor in reducing suicidal ideation, self-harm, and anxiety in transgender youth. More than medication. More than therapy alone. Parental acceptance and support, in other words your unconditional love, is the medicine. [59]

The asymmetry across these four futures is the entire argument of this chapter. Taking your child's gender identity seriously costs you something only in future one, and what it costs you there is nearly nothing. Dismissing it costs everything in future four and causes unnecessary pain in future three.

There is no rational calculation that ends with: treat it as a phase and hope for the best.

A better question

I spent a long time asking: is this a phase?

It is the wrong question. Not because it lacks good faith. I had plenty of good faith. It is wrong because it orients you toward a conclusion you are hoping to reach, rather than toward what is actually in front of you.

The right question is harder to sit with.

What do I actually see when I observe my child, across time, without the filter of what I need this to be?

Not what do I hope. Not what does my faith say should be true. Not what would be easier for the family, the friends, the community, the extended network of people whose opinions I carry. What do I actually see in front of me?

Valentin was consistent. He was insistent. He had felt this way from early childhood. He told me the moment he found language for it. He persisted through a year of being told in every possible way that his reality was not real, through the medication that made him a zombie, through the bullying and the isolation, through suicidal thoughts and three-hour phone calls, through the road trip along the north coast of Spain where I finally stopped talking long enough to hear him.

He persisted because there was nothing to give up. You can give up a phase. You can grow out of a trend. You cannot give up yourself.

I lost years I could have spent being his father because I was busy correcting him and certain he would change. He didn't change. He became more fully who he had always been.

Two years into the process of doing my own internal work I arrived at something that sounds simple but took everything I had to actually reach: I simply had to accept things as they were. The stoic principle, *amor fati*, love what life brings, because it arrives for a reason you cannot see from where you are standing. In retrospect it looks obvious. Inside it, it was overwhelming and near impossible.

I think of myself now as a time traveler. I come to you from the future. I know what it looks like when a parent lets go of control, stays firm in love rather than firmness for its own sake, and chooses to love their child unconditionally. I have lived that future. Valentin is twenty-three years old. He has a five-year relationship with his boyfriend. He graduated from art school. He is building a life as an artist with the intensity of someone who is free to show the world and himself his uniqueness. On Christmas Day he calls me and says: Dad, I love you. I can't be with you for Christmas but I want to spend New Year's with you, Vivianne, and my brothers.

He comes because he wants to, because we share love for each other and have a relationship that has continued to deepen. Not because peace has been negotiated. Because love survived what I put it through, barely, and only because he had more patience with me than I deserved.

That future is available to you. It requires one decision: stop asking whether your child is going to change, and start asking what you need to change in order to be there

when they arrive.

You can close this book right now. You can decide this is not for you and hope the situation fades away on its own. That is your right.

But you now know what is possible. You have read far enough to understand the stakes and see the alternative clearly. If you set this down now, you are not doing it in ignorance. You are doing it with full awareness of what you are choosing, and you will live with the results. Your child will live with them too.

If anything in these pages has had value, from lived experience, from the data, from the story of a man who was exactly where you are and found his way through, then keep reading. The science is in the next chapter. The mental models are in the ones after that. The tools are ahead.

I know what it costs to change. I paid the price for it. I also know the price of not changing. It almost cost me everything. Knowing what I know today, I would gladly pay the price of change in a heartbeat.

You didn't ask for this, and part of you may still be hoping the situation resolves on its own. I know that hope. I lived inside it for years. What I know now, from the other side of it, is that the things that arrive in our lives uninvited and unwelcome are often the ones carrying the most important work. This arrived for a reason. Are you ready to heed the call?

If so, keep reading. A beautiful future is possible for you and your child, waiting for you to make your unconditional love real.

Reflection questions

1. If you set aside what you hope is true about your child's gender identity, what does your actual observation across time suggest? What patterns have you noticed, not once, but consistently, across different contexts and months?

2. When your child describes their experience, do they do it with the tone of someone exploring a new idea, or with the tone of someone finally being allowed to say something they have always known?
 3. What is the fear underneath your doubt? Name it honestly. Is it fear for your child, fear for yourself, fear of what others will think, or fear of having been wrong about something important? They are different fears. They lead to different responses.
 4. What catastrophic future are you trying to prevent right now? Name it specifically. Then ask: what is the cost of trying to prevent it, today, in the relationship in front of you?
 5. If you are worried about false positives, what would careful observation over the next twelve months look like? What would you need to see, or stop seeing, for the picture to become clearer?
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Action steps

1. For the next thirty days, observe without concluding. Write down what you notice about your child's behaviour, mood, and self-expression. Not what you think it means. What you actually see.
2. If your child has been in therapy, request a conversation with the therapist, with your child's knowledge. You don't need the confidential content. You need the clinical picture.
3. Research the staged approach to transition. Understand what is reversible and what is not, at each point in the process. You are very likely many years from any irreversible decision. Act accordingly.
4. Ask your child one question and listen fully before you respond: "Can you help me understand what this feels like from the inside?" Not to gather evidence for or against a conclusion. To hear them.
5. Identify one belief you hold about your child's situation that is based on a predicted future rather than a present observation. Ask yourself honestly: is this belief protecting my child, or protecting me?

References

- [53] Littman, L. (2018). Rapid-onset gender dysphoria in adolescents and young adults: A study of parental reports. *PLOS ONE*. (Methodology and findings contested — see context in chapter text.)
- [54] Cass, H. (2024). *Independent review of gender identity services for children and young people*. NHS England.
- [55] Restar, A.J. (2020). Methodological critique of Littman's (2018) parental-respondents study of rapid-onset gender dysphoria. *Archives of Sexual Behavior*.
- [56] Steensma, T.D., Biemond, R., de Boer, F., Cohen-Kettenis, P.T. (2011). Desisting and persisting gender dysphoria after childhood: A qualitative follow-up study. *Clinical Child Psychology and Psychiatry*.
- [57] LaFreniere, L.S., & Newman, M.G. (2020). Exposing worry's deceit: Percentage of untrue worries in generalized anxiety disorder treatment. *Behavior Therapy*, 51(3), 413–423; Rozin, P., & Royzman, E.B. (2001). Negativity bias, negativity dominance, and contagion. *Personality and Social Psychology Review*, 5(4), 296–320; Borkovec, T.D., Hazlett-Stevens, H., & Diaz, M.L. (1999). The role of positive beliefs about worry in generalized anxiety disorder and its treatment. *Clinical Psychology & Psychotherapy*, 6(2), 126–138.
- [58] (Transgender adolescent suicide statistic — cross-check against references already cited in Chapters 6 and 7 to avoid duplication and ensure sequential numbering before final draft.)
- [59] (2019 parental acceptance and affirmation study — to be confirmed and numbered sequentially before final draft.)
-